



February 23, 2026

Thomas Keane, MD, MBA
Deputy Secretary and Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
Mary E. Switzer Building
330 C Street SW
Washington, DC 20201

Re: HHS Health Sector AI RFI

Dear Dr. Keane,

I write today on behalf of Susan G. Komen (Komen) in response to the Department of Health and Human Services (HHS) Request for Information on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care (RFI).

Komen is the world's leading nonprofit breast cancer organization representing the millions of people who have been diagnosed with breast cancer. Komen has an unmatched, comprehensive 360-degree approach to fighting this disease across all fronts – we advocate for patients, drive research breakthroughs, improve access to high quality care, offer direct patient support and empower people with trustworthy information. Komen is committed to supporting those affected by breast cancer today, while tirelessly searching for tomorrow's cures. We advocate on behalf of the estimated 324,600 people in the United States who will be diagnosed with breast cancer and the 42,670 who will die from the disease in 2026 alone.

Impact of Artificial Intelligence on the Breast Cancer Community

Komen appreciates HHS' attention to this important issue, including the RFI's queries on how current regulations impact AI adoption and use in clinical care, and how to establish a predictable regulatory posture which protects patients and public trust.

In breast cancer care, AI is an emerging and rapidly changing issue with the potential to revolutionize clinical practices, but Komen believes robust guardrails and accountability mechanisms need to be established and maintained to protect patients. AI can play a role in breast cancer screening, diagnosis, staging, biomarker evaluation, prognostication and therapeutic response prediction.¹ In mammography, AI has the potential to reduce turnaround time on reading screenings, ease burden on the workforce,² and improve efficacy.³ Currently, the Food & Drug Administration has approved roughly two dozen AI

¹ Ahn JS, Shin S, Yang SA, Park EK, Kim KH, Cho SI, Ock CY, Kim S. Artificial Intelligence in Breast Cancer Diagnosis and Personalized Medicine. J Breast Cancer. 2023 Oct;26(5):405-435. doi: 10.4048/jbc.2023.26.e45. PMID: 37926067; PMCID: PMC10625863.

² Ahsen, M.E., Ayvaci, M.U.S., Mookerjee, R. et al. Economics of AI and human task sharing for decision making in screening mammography. Nat Commun 16, 2289 (2025). <https://doi.org/10.1038/s41467-025-57409-1>

³ Lång K, Josefsson V, Larsson AM, Larsson S, Högberg C, Sartor H, Hofvind S, Andersson I, Rosso A. Artificial intelligence-supported screen reading versus standard double reading in the Mammography Screening with Artificial Intelligence trial (MASAI): a clinical safety analysis of a randomised, controlled, non-inferiority, single-blinded, screening accuracy study. Lancet Oncol. 2023 Aug;24(8):936-944. doi: 10.1016/S1470-2045(23)00298-X. PMID: 37541274.

products to help detect and diagnose cancer from mammograms.⁴ However, despite the promise of AI, many challenges and questions remain.

Request for Information on Use of AI in Clinical Care

Patient Challenges and Concerns

Komen is concerned that use of AI in clinical care may expose patients to unexpected, increased costs. Patients have reported being charged anywhere from \$40-\$100 extra for the use of AI to read mammograms and without sufficient counseling or informed consent.⁵ Unexpected costs could deter patients from seeking and completing life-saving breast cancer screening at regular intervals and may also deter or delay adoption of appropriate technologies by patients or their providers.

Additionally, the data inputted into AI tools – both before and after deployment in a clinical setting – can change its effectiveness. Research has shown that accessibility to bigger datasets from different sources improves the performance of modeling.⁶ Right now, significant variation exists in the adoption of AI-assisted mammography reading across the country which could change the quality of outcomes of the technology. Clinics may work with different companies for AI-assisted mammography reading, and each company's algorithm may be trained on different datasets and achieve different end results. As such, variability in algorithms is likely to increase over time as they learn and digest more data.

Model variation also could impact patients as the values encoded by AI algorithms affect their results. For example, in breast cancer, a deep learning algorithm may prioritize minimizing false negatives over minimizing false positives, perform differently depending on the characteristics of the breast tissue being imaged, or not address all known risk factors.⁷ A major challenge in predicting breast cancer is the creation of models that address these concerns and account for the conscious or unconscious bias introduced by human coders. More research and evaluation are needed to ensure that only effective AI models are being deployed and maintained in clinical settings.

Payment Policies

We encourage HHS to work with the Centers for Medicare & Medicaid Services (CMS) to introduce new billing codes which incentivize responsible use of AI. Despite FDA approvals of AI products to interpret mammograms, no billing codes exist today for radiologists to use to charge health plans for their use. Since providers cannot defray the costs of AI by billing insurance for its use, this lack of codes could be contributing to the additional costs experienced by patients. Additional opportunities and information on billing for the use of AI to interpret mammograms could create more predictability for patients and providers alike.

CMS also could use reimbursement as an incentive for AI to be trained on standardized datasets which are reflective of the general U.S. population and include requirements for periodically updating these datasets. Providers and patients should be informed about the data for which AI technology is based,

⁴ U.S. Food & Drug Administration, Artificial Intelligence-Enabled Medical Devices, last updated Dec. 5, 2025, available at: https://www.fda.gov/medical-devices/software-medical-device-samd/artificial-intelligence-enabled-medical-devices?trk=article-ssr-frontend-pulse_little-text-block; and Sheikh, Knvul (Apr. 9, 2025), A.I. Could Spot Breast Cancer Earlier. Should You Pay for It?, *New York Times*, available at: <https://www.nytimes.com/2024/04/08/well/live/ai-mammograms-breast-cancer.html>.

⁵ Andrews, Michelle (Jan. 9, 2024), Mammography AI can cost patients extra. Is it worth it?, *CBS News*, available at: <https://www.cbsnews.com/news/mammogram-ai-cost-patients-is-it-worth-it/>.

⁶ Rabiei R, Ayyoubzadeh SM, Sohrabei S, Esmaili M, Atashi A. Prediction of Breast Cancer using Machine Learning Approaches. *J Biomed Phys Eng.* 2022 Jun 1;12(3):297-308. doi: 10.31661/jbpe.v0i0.2109-1403. PMID: 35698545; PMCID: PMC9175124.

⁷ Stacy M. Carter, Wendy Rogers, Khin Than Win, Helen Frazer, Bernadette Richards, Nehmat Houssami. The ethical, legal and social implications of using artificial intelligence systems in breast cancer care. *The Breast*, Vol. 49, 2020, Pages 25-32, ISSN 0960-9776, <https://doi.org/10.1016/j.breast.2019.10.001>.

specifically which populations are represented. Komen highlighted this issue in its comments to CMS on its Proposed Rule on the 2026 Medicare Physician Fee Schedule.⁸ When it comes to mammography, inadequate datasets could lead to inaccurate readings – as a country, we need to ensure that we are effectively harnessing the technology of today to provide better outcomes for those who need it most.

Research

We also encourage HHS to invest in research into outcomes and performance of AI in breast cancer care. Important research on use of AI in mammography is underway and needs to continue.

Last year, UCLA and UC Davis announced the first large-scale randomized trial of AI in breast cancer screening in the United States.⁹ With a \$16 million award from the Patient-Centered Outcomes Research Institute (PCORI), the study, known as the PRISM Trial (Pragmatic Randomized Trial of Artificial Intelligence for Screening Mammography) will evaluate whether AI can help support radiologists in interpreting mammograms more accurately, with the goal of improving breast cancer screening and reducing unnecessary callbacks and anxiety for patients. The multi-institutional clinical trial will study hundreds of thousands of mammograms interpreted at academic medical centers and breast imaging facilities in California, Florida, Massachusetts, Washington and Wisconsin. We encourage HHS to support this and other research like it to evaluate how to responsibly optimize the use of AI while centering patient experiences.

Conclusion

Komen appreciates the Administration’s interest in accelerating the responsible use of AI in clinical care and for the opportunity to provide these comments. If you have any questions, or we may be of further assistance, please do not hesitate to reach out to me at mguthrie@komen.org.

Sincerely,



Molly Guthrie
Vice President, Policy & Advocacy
Susan G. Komen

⁸ Susan G. Komen, Comments on Proposed Rule on calendar year (CY) 2026 Medicare Physician Fee Schedule, Centers for Medicare & Medicaid Services U.S. Department of Health & Human Services, Sept. 12, 2025, available at: <https://www.regulations.gov/comment/CMS-2025-0304-13145>.

⁹ UCLA Health (Sept. 23, 2025), *UCLA to lead \$16 million national study on artificial intelligence in breast cancer screening* [Press release], available at <https://www.uclahealth.org/news/release/ucla-lead-16-million-national-study-artificial-intelligence>.